

RIVERSIDE NEUROLOGY ASSOCIATES

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CONSULTATION NOTE

PATIENT INFORMATION

Patient:	Maria Elena Santos	DOB:	03/14/1988
Date of Service:	February 12, 2026	Visit Type:	Consultation
Referring MD:	Dr. Jennifer Walsh	Insurance:	Hartford WC
Physician:	Sarah Goldstein, MD		

REASON FOR CONSULTATION

Evaluation of concussion following workplace fall on 02/08/2026. Referred from ER for post-concussion management.

HISTORY OF PRESENT ILLNESS

Ms. Santos is a 35-year-old right-handed female carpenter who sustained a head injury when she fell approximately 15 feet from scaffolding on February 8, 2026. She struck her head on the concrete floor, resulting in a 3cm forehead laceration that was sutured in the ER. CT head at the time was negative for intracranial hemorrhage or skull fracture.

Current Symptoms (4 days post-injury):

- Headache: Constant, described as pressure-like, rated 5/10
- Dizziness: Intermittent, worse with position changes
- Difficulty concentrating: “Can’t focus on reading or TV”
- Fatigue: “I sleep 12 hours and still feel tired”
- Photophobia: Mild sensitivity to bright lights
- Irritability: Reports feeling “on edge”
- Memory: Some difficulty with short-term recall

She denies loss of consciousness at the scene (per witness reports she was awake immediately after the fall, though dazed). She denies nausea/vomiting, vision changes, weakness, numbness, or seizure activity.

PAST MEDICAL HISTORY

No prior head injuries or concussions. No history of migraines. No neurological conditions.

MEDICATIONS

- Norco 5/325mg PRN (taking 1-2x daily)
- Ibuprofen 600mg TID
- Flexeril 10mg QHS

PHYSICAL EXAMINATION

Vital Signs: BP 128/78, HR 72, RR 14

General: Alert, oriented x3, appears fatigued.

Head: Sutured laceration to right forehead healing well. No CSF leak. No Battle's sign or raccoon eyes.

Cranial Nerves:

- II: Visual acuity 20/20 OU, visual fields full, pupils equal and reactive
- III, IV, VI: Extraocular movements intact, no nystagmus
- V: Sensation intact V1-V3 bilaterally
- VII: Face symmetric
- VIII: Hearing grossly intact bilaterally
- IX, X, XI, XII: Intact

Motor: Strength 5/5 throughout (limited testing of RUE due to wrist fracture in splint)

Sensory: Intact to light touch throughout

Coordination: Finger-to-nose intact bilaterally. Heel-to-shin intact.

Gait: Slightly unsteady, narrow-based. No frank ataxia.

Cognitive Screen (MOCA): 24/30 (Mild impairment)

- Lost points: Delayed recall (2), Attention/serial 7s (2), Abstraction (1), Orientation (1)

ASSESSMENT

1. **Post-Concussion Syndrome** - Patient exhibits multiple symptoms consistent with mild traumatic brain injury including headache, cognitive difficulties, fatigue, and vestibular symptoms. MOCA score of 24/30 supports mild cognitive impairment.
2. **Workplace injury** - Mechanism consistent with concussive injury.

PLAN

1. **Cognitive Rest:** Limit screen time, reading, and mentally demanding activities for the next 1-2 weeks. Gradually increase activity as tolerated.
2. **Physical Rest:** No strenuous activity. Light walking is acceptable. No return to work until cleared.
3. **Sleep Hygiene:** Maintain regular sleep schedule. It is normal to need extra sleep during recovery.
4. **Headache Management:**
 - Continue ibuprofen as needed but avoid overuse (rebound headache risk)
 - May add acetaminophen 1000mg as needed
 - Consider Tylenol #3 for severe headaches
5. **Return Precautions:** Return immediately or go to ER for: severe headache, repeated vomiting, seizures, confusion, slurred speech, weakness on one side, unequal pupils.
6. **Driving:** No driving until symptoms significantly improved and cleared by this office.
7. **Work Status: TOTALLY DISABLED** from all work activities. Patient cannot safely perform job duties or operate machinery.
8. **Follow-up:** Return in 2 weeks for re-evaluation. If symptoms persist beyond 4 weeks, will consider neuropsychological testing and vestibular rehabilitation referral.

9. **Prognosis:** Good. Majority of concussion symptoms resolve within 2-4 weeks. Given mechanism and initial presentation, anticipate full recovery though timeline may be prolonged due to concurrent injuries.

Sarah Goldstein, MD
Board Certified Neurology
Riverside Neurology Associates

Date: February 12, 2026